QUÉTIAPINE

Xeroquel® · Séroquel® · Dibenzothiazépine · Antipsychotique atypique · Éq. CPZ ≈ 75 mg

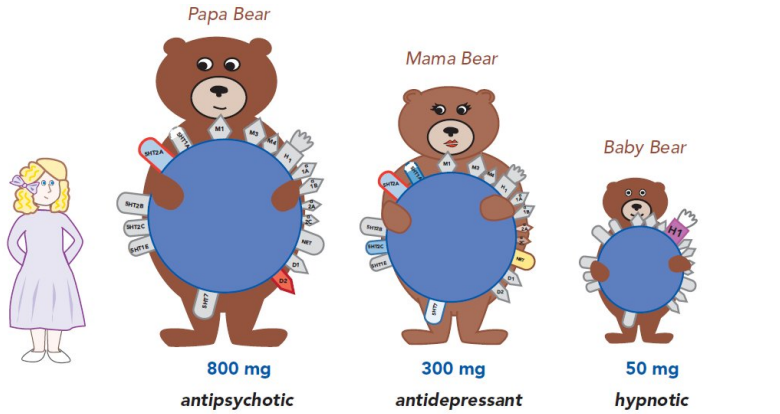

Figure 5-46 Binding profile of quetiapine at different doses. The binding properties of quetiapine vary depending on the dose used. At antipsychotic doses (i.e., up to 800 mg/day), quetiapine has a relatively wide binding profile, with actions at multiple serotonergic, muscarinic, and α-adrenergic receptors. Histamine 1 receptor blockade is also present. At antidepressant doses (i.e., approximately 300 mg/day), the binding profile of quetiapine is more selective and includes norepinephrine reuptake inhibition, $5HT_{1A}$ partial agonism, and $5HT_{2A}$, α_2, $5HT_{2C}$, and $5HT_7$ antagonism. At sedative hypnotic doses (i.e., 50 mg/day), the most prominent pharmacological property of quetiapine is H_1 antagonism.

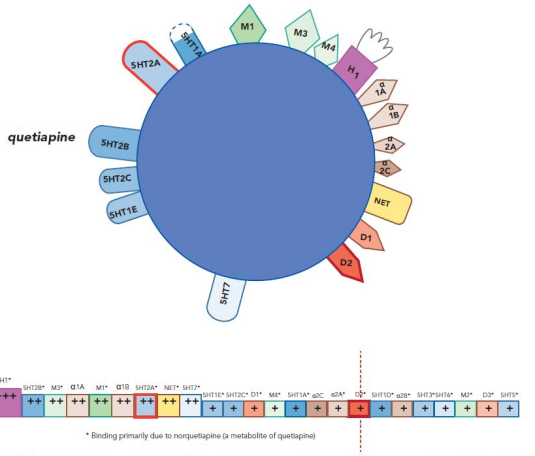

Figure 5-45 **Quetiapine's pharmacological and binding profile.** This figure portrays a qualitative consensus of current thinking about the binding properties of quetiapine. Quetiapine does not actually have particularly potent binding at D_2 receptors. Quetiapine's prominent H_1 antagonist properties probably contribute to its ability to enhance sleep, and this may contribute as well to its ability to improve sleep disturbances in bipolar and unipolar depression as well as in anxiety disorders. However, this property can also contribute to daytime sedation, especially combined with M_1 antimuscarinic and α_1-adrenergic antagonist properties. A potentially important active metabolite of quetiapine, norquetiapine, may contribute additional actions at receptors, as noted in the binding profile with an asterisk. $5HT_{1A}$ partial agonist actions, norepinephrine transporter (NET) inhibition, and $5HT_{2C}$, α_2, and $5HT_7$ antagonist actions may all contribute to mood-improving properties of quetiapine. However, $5HT_{2C}$ antagonist actions combined with H_1 antagonist actions may contribute to weight gain. As with all agents discussed in this chapter, binding properties vary greatly with technique and from one laboratory to another; they are constantly being revised and updated.

PROFIL PHARMACOLOGIQUE

Classe	Dibenzothiazépine — antipsychotique atypique (SGA)
Mécanisme principal	Antagoniste D2, 5-HT2A, H1, α1, M — profil **dose-dépendant** (voir figures)
Métabolite actif	Norquétiapine (N-désalkyl-quétiapine) — contribue au blocage NET, agonisme 5-HT1A partiel
Éq. chlorpromazine	≈ 75 mg CPZ pour 100 mg quétiapine · Équivalence 1:1,3
Pharmacocinétique	t½ : 6–7 h (quétiapine) / 9–12 h (norquétiapine) · Pic plasma 1–2 h · Métabolisme CYP3A4 ++

INDICATIONS ET POSOLOGIES · AMM France

Indication (AMM)	Posologie cible	Remarques
Schizophrénie	400–800 mg/j (max 800)	LP ou IR · Profil sédatif utile en phase aiguë · Titration sur 4 j
Épisode maniaque bipolaire	400–800 mg/j	Titration rapide possible en hospitalisation
Dépression bipolaire	300 mg/j (dose-cible)	AMM spécifique · Profil antidépresseur via norquétiapine + 5-HT1A
Prévention rechutes bipolaire	300–800 mg/j	Forme LP privilégiée pour l'observance
Insomnie / sédation (hors AMM)	25–100 mg au coucher	Action H1 prédominante — usage courant mais hors AMM — dépendance possible
Dépression unipolaire en add-on (hors AMM)	50–300 mg/j	Via norquétiapine (NET + 5-HT1A) · Données probantes mais AMM non obtenue en FR

TOLÉRANCE ET POINTS DE VIGILANCE

Effets indésirables fréquents	Points de vigilance / contre-indications
Sédation +++ (H1) — surtout en début de traitement	QTc : allongement possible — ECG si FR ou association
Prise de poids / syndrome métabolique (H1 + 5-HT2C)	Hypotension orthostatique (α1) — titration lente recommandée
Hypotension orthostatique (α1) — surtout initiation	CYP3A4 : inducteurs (carbamazépine) ↓ taux × 6 — éviter
Sécheresse buccale, constipation (M)	Grossesse : données limitées — rapport bénéfice/risque
Hyperglycémie, dyslipidémie (suivi biologique trimestriel)	Sevrage brutal : insomnie rebond, nausées — décroissance progressive
SPC rares (faible affinité D2) · Pas d'hyperprolactinémie significative	Personnes âgées : risque AVC/mortalité (psychose démence) — prudence

Formes disponibles (FR) :	Xeroquel® / Séroquel® IR : cp 25, 100, 200, 300 mg	Xeroquel® / Séroquel® XR (LP) : cp 50, 150, 200, 300, 400 mg	Génériques disponibles

Sources : Stahl's Essential Psychopharmacology (Figures 5-45 & 5-46) · Fiches Vidal / HAS · ANSM · Usage clinique ; ne se substitue pas aux RCP officiels.